

العمر(تاريخ الميلاد) : 13-01-1973 المهنة : حاصل على بكالوريوس

رئيس
المجلس الطبي المتخصص

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Name: مرفت نبيل محمد ابراهيم National ID: 27301130101161	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name: Mervat Nabil Mohammed
Date: 2026-06-17
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA underwent right MRM.

I- CHEST:

- Small right middle lobe pleural based pulmonary nodule measuring 6.8 mm.
- Few bilateral scattered fine atelectatic bands.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Right hepatic lobe segment VII hypodense focal lesion measuring 13 x 11 mm. no dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulæ or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few sacral, right iliac bone and left femoral head lytic bone lesions.

Opinion:

- ❖ Small right middle lobe pulmonary nodule for follow up.
- ❖ Right hepatic lobe focal lesion for dynamic MR study.
- ❖ Few sacral, right iliac bone and left femoral head lytic bone lesions likely metastatic for bone scan.

Dr.sherif Wagdy

Dr.sherif Wagdy

Name : Mervat Nabil Mohammed Ibrahim
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 29-06-2026 09:46
Medical No. : 405021

Tc-99m MDP Whole-Body Bone Scintigraphy

Comparison: Prior bone scan dated 09/07/2025

Clinical History: A 52-year-old female patient with a known history of right breast carcinoma managed by total mastectomy, adjuvant chemotherapy, and radiotherapy, currently on hormonal therapy and presents for follow-up evaluation.

Technique: Following the intravenous injection of Tc-99m MDP, anterior and posterior whole-body planar images were acquired approximately 2 hours post-injection.

Findings:

- Known sacrum/right SI and left femoral neck metastases show stable to minor regression.
- Old sternal lesion (retrospectively noted) similarly stable/regressive.
- No new suspicious foci.
- Benign age-appropriate degenerative spinal/joint uptake.
- Normal soft tissues and renal excretion.

Impression:

- Stable to regressive known skeletal metastases. No new lesions.

Recommendation:

- Continue follow-up.
- Consider CT/MRI of left femoral neck for cortical stability monitoring.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

جمهورية مصر العربية
بطاقة تحقيق الشخصية



مرفق
نبيل محمد ابراهيم
٣١ ش عفيفي مصطفى الشاهد - ش السودان
العجوزه - الجيزه

٢٧٣.٠١ ١٣.٠١ ١١ ٦١



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IK3260933

DMS

تدريس



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حاصلة على بكالوريوس دراسات تعاونية



أنثى مسلمة متزوجة
اسمها عبد الحميد عبد الرحيم سيد احمد
البطاقة سارية حتى ٢٠٢٧/٠٦/٠٥

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MDT FORM ١٩ / ٨ / 2022 6

Name	أ/م/ب/ج/د/هـ	MRN	0000/0000
Age	52 y.	ID	
Menopausal status	premenop.	Phone	01003017905
NRS SCORE		Marital status	CV 113.1171
BMI		Family history	
CO morbidity		REASON OF MDT	Newly developp. Bone Met.

Clinical Data

BX:	LOC	R	PSIT	No visible Cm.
ER:	PR:	HER 2:	KI67:	
8/8	5/8	-	15/	

Sonomammography: premenop. Breast cancer
with Newly developed Bone Met.

Mets Workup: Histy: Rt Breast cancer locally adv. ^{TNM}
L NACT on 20/6/2022
Rt HRH 4pT2 + 4PN2 9/15

Recommendations: Rec. adjuvant Femora + Zoladex f. 8/2022
till Now.

Readdy Boypr:
Bone scan 9/7/2022: worrisome osseous les
at Rt ST joint, sacrum, Neck of Left femur

MDT OUTCOME & PLAN: Likely Metastases.
Fasoldex / CDK4/6 - 1/11/22 - Rth 300X10 - Zoladex
Jr. MDT 2/11/22 HRT 15K - 10/11/22

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. Kader	Dr. Hoss 19/8/2022				

Name : Mervat Nabil Mohamed Ibrahim

Date : 18-12-2025 02:36

Branch : Portable

Medical No. : GRI-13664

Ref.Physician Prof :

Ins.Company :

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS:

Findings: Right mastectomy, compared to 2/10/2025

I-CHEST:

Right mastectomy with clear operative bed, no masses or collection.

- Clear left breast and both axillae.
- Right upper and middle lobe subpleural reticulations likely post radiotherapy.
- Clear both lung fields.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is mildly enlarged in size with homogenous CT texture. Regressive course as regards the size of the multiple bilobar hepatic focal lesions , two are seen at segment III measures 13x11mm compared to 19 x17 mm and 8x7mm compared to 13 x6 mm in previous study, another FL seen at segment VIII measures 13x12mm compared to 22x 21 mm ,resolution of focal lesions at segment I and VI.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

Name : Mervat Nabil Mohamed Ibrahim
 Branch : Portable
 Ref.Physician Prof :

Date : 18-12-2025 02:36
 Medical No. : GRI-13664
 Ins.Company :

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes .
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Mild diverticular disease of sigmoid and descending colon.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

III The scanned skeleton:

- Stationary course as regards:
- Right sacral destructive bony lesion with associated soft tissue component measures 5.5 x3 cm
- Large lytic lesion of left femoral head.
- No vertebral structural collapse.

Opinion:

- Right mastectomy with clear operative bed.
- Regressive course as regards the size and the number of multiple bilobar hepatic focal lesions, likely metastatic.
- Stationary course as regards the osseous metastasis.

Dr. Mai Shaaban MD

Dr. Mai Shaaban

Patient Name: Mervat Nabil Mohamed Ibrahim	Date : 02/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 5254/2021	Pat. ID: 13664

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST , ABDOMEN & PELVIS : -

Findings: right mastectomy

I-CHEST :

Right mastectomy with clear operative bed , no masses or collection.

- Prominent left level I axillary LNs.
- Right upper and middle lobe subpleural reticulation likely post radiotherapy
- Clear both lung fields
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.

II Abdomen & pelvis:

- The liver is mildly enlarged in size with homogenous CT texture. Multiple bilobar hepatic focal lesions , two are seen at segment III measures 19 x17 mm and 13 x6 mm , another FL seen at segment VIII measures 22 x21 mm ,segment I measures 6 x5 mm& small lesion 10 x 5 mm seen at segment VI.they show faint contrast enhancement.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back pressure changes .
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Diverticular disease of sigmoid and descending colon.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

III The scanned skeleton:

- Right sacral destructive bony lesion with associated soft tissue component measures 5.1 x3 cm
- Large lytic lesion of left femoral head.
- No vertebral structural collapse .

Opinion:

- *Right mastectomy with clear operative bed .*
- *Right upper lobe subpleural reticulation likely post radiotherapy .*
- *Multiple bilobar hepatic focal lesions ,likely metastatic .*
- *Large sacral destructive lesion with soft tissue component .*
- *Left femoral lytic lesion .*

Dr. Reham Khedr, ML

Patient Name: Mervat Nabil Mohamed Ibrahim	Date : 02/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 5254/2021	Pat. ID: 13664

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS:

PELVIS:

Findings: Right mastectomy

I-CHEST:

Right mastectomy with clear operative bed , no masses or collection.

- Prominent left level I axillary LNs.
- Right upper and middle lobe subpleural resticulation likely post radiotherapy.
- Clear both lung fields.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
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- Large lytic lesion of left femoral head.
- No vertebral structural collapse .



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بطاقة تحريق الشخصية

مرفت
نبيل محمد ابراهيم

٣١ ش عفيفي مصطفى الشاهد - ش السودان
العجوزه - الجيزه

١٩٧٣/٠١/١١ ٢٧٣.٠١١٣.٠١.١١ ٦١

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